MSC MARRIAGE AND FAMILY THERAPY

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MFT 720

Supervision and Consultation in Therapy

15 teaching weeks · 1 figure

SEMESTER THREE

1.71 units · Prerequisite: MFT 706, 704

> “Supervision is the profession's quality-control mechanism and its main method of training.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

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MFT 720 — Supervision and Consultation in Therapy

MSc Marriage and Family Therapy · Semester Three · 1.71 Units · 15 teaching weeks · Prerequisite: MFT 706, 704

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	**Supervision and Consultation in Therapy**	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. Supervision is the profession's quality-control mechanism and its main method of training. This course prepares you to receive supervision well and to give it competently — with models, structure, feedback skills, attention to power, and the discipline of parallel process.

How to use this book

You are simultaneously a supervisee and a developing supervisor in this course. Each week you will analyse your own supervision (what worked, what did not, what you did not say) and practise supervising a peer. The two roles teach each other.

1. Course description

The course covers models of supervision (developmental, systemic, competency-based, reflective), the supervisory alliance, contracting and boundaries, live and recorded supervision, feedback, evaluation and gatekeeping, power and culture in supervision, parallel process, ethical and legal responsibilities, and consultation to organisations.

2. Learning outcomes

#	Outcome	Assessed
LO1	Describe and apply at least three supervision models	Exam
LO2	Contract clearly with a supervisee, including evaluation criteria	Contract document
LO3	Give specific, behavioural feedback that changes practice	Observed supervision
LO4	Identify and use parallel process	Case analysis
LO5	Manage power, culture and conflict in supervision	Reflective portfolio
LO6	Fulfil gatekeeping responsibilities ethically	Viva
LO7	Consult to a team or organisation	Consultation report

3. Assessment

Component	Weight	Due
Supervision contract and learning plan	15%	Week 4
Observed supervision session (40 min)	30%	Week 10
Case analysis of parallel process (1,500 words)	20%	Week 12
Consultation report (2,000 words)	25%	Week 14
Reflective portfolio	10%	Ongoing

4. The fifteen-week plan

Week	Topic	Practical
1	What supervision is for; supervision versus therapy versus mentoring	Role clarification
2	Models I: developmental and competency-based	Model application
3	Models II: systemic and reflective	Reflecting team in supervision
4	**Contract due**; contracting and boundaries	Contract negotiation
5	The supervisory alliance and rupture repair	Alliance repair rehearsal
6	Structure: agenda, case presentation formats, time	Structured case presentation
7	Feedback that changes practice	Feedback lab
8	Live supervision and recorded review	Live observation
9	Parallel process and the supervision triangle	Parallel process analysis
10	**Observed supervision session**	Assessed
11	Power, culture, gender and race in supervision	Power audit
12	**Parallel process analysis due**; conflict and incompetence	Difficult conversations
13	Evaluation, gatekeeping and fitness to practise	Evaluation interview
14	**Consultation report due**; consultation to organisations	Consultation simulation
15	Integration: the supervisor you want to be	Viva

Week 1 — What supervision is for

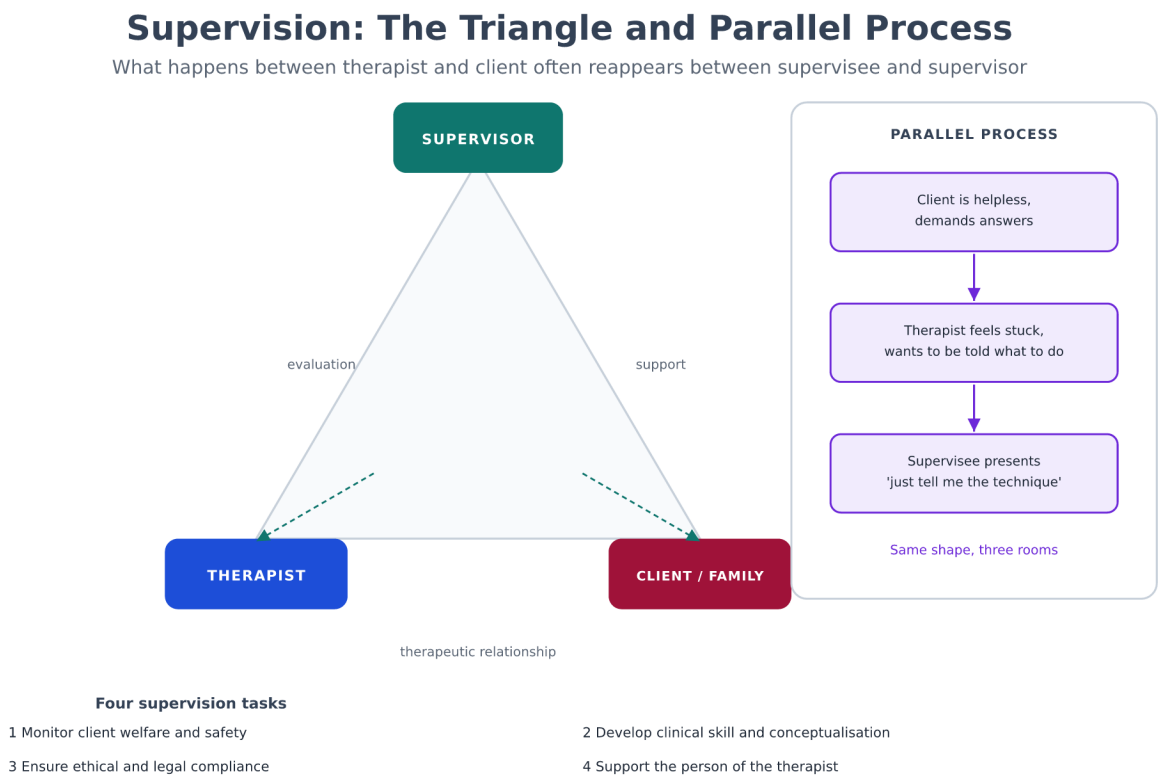


Figure 1.1 — The supervision triangle and parallel process

Four functions (Kadushin, adapted).

1. **Clinical** — ensuring the client receives good, safe care.
2. **Developmental** — building the supervisee's competence.
3. **Supportive** — sustaining the person doing difficult work.
4. **Gatekeeping** — protecting the public and the profession.

What supervision is not. It is not therapy, although it may be therapeutic. It is not mentoring, which is about career. It is not line management, although it overlaps. Confusing these roles is the most common source of boundary difficulty.

The client is the third party in the room. Whatever the supervisee needs, the question that governs the supervision is: is this client safe and well served?

Week 2 — Developmental and competency models

Developmental models (e.g. Stoltenberg) describe stages: the supervisee moves from dependence and anxiety, through dependence–autonomy conflict, to conditional autonomy, to mastery. The supervisor's style must change with the stage: more structure early, more consultation later.

Stage	Supervisee needs	Supervisor stance
Novice	Structure, direct guidance, safety	Teaching, modelling, clear expectations
Advanced beginner	Autonomy with support	Coaching, prompting, joint formulation
Competent	Challenge, complexity	Consulting, debating, opening alternatives
Proficient	Peer-level dialogue	Collegial, focusing on the system and self

Competency-based models specify observable skills and rate them, using frameworks such as the UCL competence frameworks. Their strength is transparency and defensibility; their risk is reducing supervision to a checklist.

The best use of both. Use developmental stage to decide how much structure, and competencies to decide what to look for.

Week 3 — Systemic and reflective models

Systemic supervision attends to the system of client–therapist–supervisor, and to the wider systems (agency, family, culture). The supervisor asks: what is the function of this presentation in this system, at this time?

Reflective supervision uses the supervisor's and supervisee's emotional responses as data, and models curiosity rather than expertise. Techniques include the reflecting team (in supervision), genograms of the case, and structured self-reflection.

Live supervision (behind a mirror, or in-room) gives immediate data. Formats: the phone-in, the "whispering" supervisor, the consultant who joins for ten minutes, and the reflecting team.

The advantage of live work is that it reveals what the therapist does, not what they remember doing. The disadvantage is anxiety, which must be managed explicitly in the contract.

Week 4 — Contracting

A supervision contract covers:

1. Purpose and scope; who else is involved (agency, university).
2. Frequency, duration, location, cancellation.
3. Case presentation format and preparation expectations.
4. Recording and observation policy, including consent from clients.
5. Confidentiality and its limits (client safety, fitness to practise, legal).
6. Evaluation: criteria, timing, who sees it, and consequences.
7. Boundaries: contact between sessions, personal disclosure, dual relationships.
8. Procedures for disagreement, complaint and termination.

Negotiate, do not impose. The contract should include the supervisee's goals: "By the end of this placement, I want to be able to…"

> **DOCUMENT (15%)**
> A supervision contract and learning plan for a first-placement supervisee.

Week 5 — The supervisory alliance

Three components (parallel to the therapeutic alliance): goals, tasks, and bond.

Ruptures are inevitable: a supervisee feels judged; a supervisor feels ignored; a disagreement about a case becomes personal. The repair is the model.

Repair script.

> "I noticed you went quiet when I suggested you had missed the risk. Did I get that wrong, or did it land badly?" "What I intended was a question, not a verdict. Let me say it again differently."

The unspoken curriculum. Supervisees learn more from how you behave than from what you teach. If you avoid difficult feedback, they will avoid it with clients. If you are curious about your own mistakes, they will be curious about theirs.

Week 6 — Structure

A 50-minute supervision session.

Minutes	Content
0–5	Check-in; agenda setting
5–15	Case 1: presentation, formulation, question
15–30	Case 1: exploration, recording review, options
30–40	Case 2 or a theme (risk, ethics, culture)
40–47	Learning points; tasks for the week
47–50	Administrative and evaluation notes

A case presentation format (five minutes, no notes).

1. Who is the family, and who is in the room?
2. What is the presenting problem, in their words?
3. What is my formulation and my uncertainty?
4. What did I do, and what happened?
5. What is my question for supervision?

The discipline matters: a supervisee who cannot present in five minutes is not yet thinking systemically.

Week 7 — Feedback

Principles. Behavioural, specific, timely, owned, and limited to what can be acted on.

The four-step structure.

1. **What I observed** ("you asked three questions in a row after she cried").
2. **What it did** (to the client, or to me as observer) ("she stopped elaborating").
3. **What to keep** ("your opening was excellent — you named the risk clearly").
4. **What to try** ("next time, try a reflection before the next question").

The banned practices. The sandwich (praise–criticism–praise), "you need to be more confident" (not behavioural), public criticism, and feedback given weeks after the event.

Receiving feedback is a skill too: listen, ask for an example, resist explaining, and decide what to use.

Week 8 — Live supervision and recording

Consent. Families must consent to recording and to observation, in writing, with a clear explanation of who will watch and how long the recording is kept.

In-session formats.

- **Phone-in:** the supervisor calls in with one or two suggestions. Risk: the therapist becomes a puppet.
- **Reflecting team:** the team talks in front of the family (or behind a mirror). The therapist chooses what to bring back. Strong evidence and strong engagement effects.
- **Consultant joins:** a senior clinician joins for ten minutes and then leaves.

Debriefing. Always debrief the supervisee's experience first, then the clinical content. The order matters: the person before the problem.

Week 9 — Parallel process

Definition. The pattern in the therapy reappears in the supervision: a helpless family produces a helpless therapist who produces a supervisor who takes over; a controlling family produces a therapist who complies and a supervisor who feels ignored.

How to use it.

1. Notice your own experience in supervision (boredom, irritation, the urge to solve).
2. Ask: is this mine, the supervisee's, or the system's?

3. Name it tentatively: "I notice I am doing a lot of the thinking here. I wonder whether that is also how it feels in the room."
4. Use it to generate a hypothesis about the family.

Caution. Parallel process is a hypothesis, not a law. Do not use it to explain away a supervisee's difficulty or your own poor supervision.

> **ANALYSIS (20%)**
> A 1,500-word analysis of a parallel process from your own supervision, with evidence and alternatives considered.

Week 10 — Observed supervision session

Forty minutes, supervising a peer with a real (anonymised) case. Assessed on: contract and structure; case presentation discipline; formulation support; feedback quality; attention to the supervisee's experience; and clear learning tasks.

Week 11 — Power, culture, gender and race

Power is structural. The supervisor evaluates, gatekeeps and holds institutional authority. It cannot be wished away; it can be made transparent.

Cultural dynamics. Differences in language, ethnicity, religion, gender, class and age shape what is said and what is withheld. A supervisee from a culture that does not openly disagree with elders may appear "unreflective" when they are being respectful.

Practical disciplines.

- Discuss difference explicitly at the start: "What should I know about how you prefer to receive feedback?"
- Invite disagreement in a form that is culturally possible: "What would you say if you disagreed?"
- Notice who speaks first, who is interrupted, and whose cases get the most time.
- Examine your own assumptions about competence.

Week 12 — Conflict, incompetence and impairment

Conflict. Address early, privately, behaviourally. Distinguish disagreement about a case (healthy) from disagreement about values (harder) from a relationship rupture (needs repair).

Incompetence. Define it against the competencies, in writing, with a development plan and a review date. Most incompetence is developmental and remediable.

Impairment. When the supervisee's own difficulties affect client care (substance use, untreated trauma, exhaustion), the response is support plus boundaries, not silence. Silence is the most common failure of supervisors and the most damaging to clients.

The conversation. "I am concerned about you, and I am also concerned about your clients. Let us talk about both."

Week 13 — Evaluation and gatekeeping

Evaluation criteria should be known in advance and behavioural.

Domain	Example indicator
Clinical competence	Can conduct a structured assessment and formulation
Professional conduct	Punctuality, record-keeping, boundaries
Reflective capacity	Uses feedback; identifies own contribution
Ethical practice	Recognises and escalates risk
Relational capacity	Builds alliances with all family members

Gatekeeping. If a supervisee is not safe, the supervisor must act, even when it is painful: extend supervision, restrict caseload, require therapy, or fail the placement with a documented rationale.

Failing well. Give clear reasons, a written plan, support, and a route to appeal. Document throughout, not only at the end.

Week 14 — Consultation to organisations

The shift. Supervision develops a person; consultation helps a system. Your client is the team, the school, the clinic or the agency.

A consultation cycle.

1. **Contract:** who is the client, what is the question, who holds authority, what will be shared?
2. **Assess the system:** structure, communication, roles, alliances, and the presenting problem's function.
3. **Feedback:** present the systemic picture, with hypotheses rather than verdicts.
4. **Intervention:** a workshop, a reflecting team, a policy change, a case conference structure.
5. **Exit and evaluate:** what changed, and how will it be sustained?

Common organisational dynamics. The identified "difficult team member" who carries the team's anxiety; the "impossible case" that holds a conflict between services; the over-worked manager who mirrors the over-worked family therapist.

> **REPORT (25%)**
> A consultation report for a team or organisation, with a systemic assessment and a proposal.

Week 15 — The supervisor you want to be

Write 700 words: your model of supervision; how you will handle power; what you will never do; the feedback you find hardest to give; and how you will know you are doing harm. Defend it in a viva.

Glossary

Competency-based supervision — supervision organised around observable, rated skills. **Gatekeeping** — the supervisor's responsibility to protect the public by preventing unsafe practice. **Live supervision** — observation of therapy as it happens, with in-session input. **Parallel process** — the reproduction of the therapy dynamic in the supervision. **Reflecting team** — a group that discusses the case in front of the family, with the therapist choosing what to bring back. **Supervisory alliance** — the goals, tasks and bond of the supervision relationship.

Self-assessment

Multiple choice

1. The client in supervision is: (a) the supervisee (b) the family receiving therapy (c) the agency (d) the university

2. Gatekeeping exists to: (a) protect the profession's reputation only (b) protect the public and the profession (c) increase pass rates (d) satisfy administrators

3. Parallel process is: (a) a proven law (b) a hypothesis about the reproduction of dynamics in supervision (c) transference (d) supervision of supervision

4. A good feedback statement is: (a) general and encouraging (b) behavioural, specific and owned (c) delayed (d) public

5. Live supervision requires: (a) no consent (b) written client consent and clear explanation (c) only supervisor consent (d) audio only

6. The sandwich technique is discouraged because: (a) it is too positive (b) it trains people to hear praise as a warning (c) it is fast (d) it is illegal

7. Developmental models suggest that novices need: (a) less structure (b) more structure and direct guidance (c) peer consultation only (d) no evaluation

8. Impairment in a supervisee should be met with: (a) silence (b) support plus clear boundaries (c) immediate dismissal (d) transfer without discussion

9. Consultation differs from supervision in that it: (a) develops a person (b) helps a system (c) evaluates students (d) is always clinical

10. A supervision contract should include: (a) only the times (b) purpose, frequency, confidentiality, evaluation and boundaries (c) the supervisor's fees only (d) the client's diagnosis

Answer key

1 b · 2 b · 3 b · 4 b · 5 b · 6 b · 7 b · 8 b · 9 b · 10 b

Short answer

1. Name the four functions of supervision.
2. Describe a five-minute case presentation format.
3. What are the four steps of behavioural feedback?
4. Give three ways power operates in supervision and one way to make it transparent.
5. What should a supervisor do when a supervisee is unsafe?

Essay (choose one, 1,800 words)

1. "Supervision is the profession's most important ethical practice." Discuss.
2. Analyse a parallel process from your own experience and what it revealed about the case.
3. Discuss the management of cultural difference in supervision, with examples.

Further reading

- Hawkins, P., & Shohet, R. (2012). Supervision in the Helping Professions (4th ed.). Open University Press.
- Stoltenberg, C. D., & McNeill, B. (2010). IDM Supervision (3rd ed.). Routledge.
- Bernard, J. M., & Goodyear, R. K. (2019). Fundamentals of Clinical Supervision (6th ed.). Pearson.
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Next: MFT 721 — Research Methodology